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/ **Practice Exam - 1**

Correct Answer

Next Question

Practice Exam - 1

Question 111 of 117



A 55-year-old man with a history of hypertrophic cardiomyopathy presents to clinic with worsening fatigue and exertional dyspnea over the past 3 months. His symptoms now limit him to walking approximately 1 block before needing to stop and rest.

Vital signs are blood pressure 100/72 mmHg, heart rate 58 bpm, respiratory rate 16 breaths/min, and oxygen saturation 99% on room air. Physical examination reveals jugular venous pressure of 7 cm H₂O, normal S1 and S2, a grade 1/6 systolic ejection murmur, no crackles, and no lower extremity edema.

Transthoracic echocardiography demonstrates LVEF 60%, maximal septal wall thickness of 25 mm, and a resting left ventricular outflow tract (LVOT) gradient of 25 mmHg that increases to 130 mmHg with Valsalva.

He has been treated with Metoprolol succinate 100 mg twice daily for the past 2 years and reports adherence without dizziness or lightheadedness.

Which of the following is the best next step in management and is supported by randomized controlled trial data?

A. Switch metoprolol to carvedilol



B. Start disopyramide



C. Start mavacamten

D. Refer for septal reduction therapy

Commentary References

Testing Point: The board-certified AHFTC specialist should recognize the role of cardiac myosin inhibitor therapy in patients with symptomatic obstructive hypertrophic cardiomyopathy who remain symptomatic despite first-line medical therapy.

Correct Answer: C. Start mavacamten

Rationale:

Mavacamten is indicated for adults with symptomatic obstructive hypertrophic cardiomyopathy who remain symptomatic despite first-line therapy with nonvasodilating beta blockers or nondihydropyridine calcium channel blockers. Clinical trials demonstrated improvement in exercise capacity, symptoms, and reduction in LVOT gradients.

This patient has persistent symptoms with severe provokable LVOT obstruction despite maximally tolerated beta-blocker therapy, making mavacamten the most appropriate next step.

Answer A: Carvedilol is not preferred because vasodilating beta blockers may worsen LVOT obstruction.

Answer B: Disopyramide remains a guideline-supported therapy for symptomatic obstructive HCM in patients with persistent symptoms despite beta-blocker therapy and remains a reasonable treatment option. However, its use is supported primarily by observational and historical data rather than contemporary randomized controlled trials. In contrast, mavacamten has demonstrated efficacy in randomized clinical trials for symptom improvement and LVOT gradient reduction.

Answer D: Septal reduction therapy remains appropriate for patients with severe symptoms refractory to optimal medical therapy, but pharmacologic escalation is appropriate first in this patient.

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